SELF-ASSESSMENT TOOL

The Patient Drop-Off Scorecard

Find where your patients disappear between enrollment and first fill. Score your program in 5 minutes. Take the results to your next leadership meeting.

$22B+
ANNUAL PSP SPEND

3–8%
UTILIZATION RATE

59%
PATIENTS UNAWARE

PatientPartner

Patient Engagement Software That Delivers Measurable ROI

patientpartner.com

Pharma invests **$22B+ annually** in patient support programs. Only **3–8% of eligible patients** use them. That's not a marketing problem — it's a program design problem, and the gap is almost always in the same place.

This scorecard helps you find it. Score your program across **7 dimensions**, identify exactly where patients are falling out, and see which gaps are costing you the most starts, adherence, and revenue.

SCORING KEY

- **1** Don't do this at all
- **2** Tried, inconsistent
- **3** Do this, could improve
- **4** Do well, with data
- **5** Genuine strength

Tip: Have 2–3 stakeholders score independently, then compare. The gaps between scores reveal blind spots.

1 Patient Awareness & Discovery

Do patients know your support program exists?

> 59% of patients report little or no knowledge of available patient support programs — even when they're enrolled in one.

/ 5 **1.1** Patients learn about our support program at the point of diagnosis or prescription, not weeks later.

/ 5 **1.2** Our program is visible on the brand.com website with a clear, low-friction entry point (not buried in a footer).

/ 5 **1.3** HCPs and office staff actively introduce patients to our support resources during the appointment.

/ 5 **1.4** We have a multi-channel awareness strategy (email, SMS, in-office, digital) rather than relying on a single touchpoint.

/ 5 **1.5** We measure awareness — we know what percentage of eligible patients are aware of and can describe our program.

SECTION 1 TOTAL / 25

2 Enrollment & Onboarding Speed

How fast do patients go from aware to enrolled?

> Every day of delay between prescription and program enrollment increases the likelihood of abandonment. A complicated enrollment process loses patients before they ever receive support.

/ 5 **2.1** A patient can enroll in our support program in under 5 minutes from any device.

/ 5 **2.2** Enrollment doesn't require a phone call, fax, or mail-in form as the primary path.

/ 5 **2.3** Patients receive a welcome touchpoint (call, text, email) within 24 hours of enrollment.

/ 5 **2.4** We track time-to-enrollment and time-to-first-contact as operational KPIs.

/ 5 **2.5** Our drop-off rate between "eligible patient identified" and "enrolled in program" is below 40%.

SECTION 2 TOTAL / 25

3 Emotional Readiness & Psychological Support

Do you address the fear, doubt, and overwhelm that cause patients to stall?

> This is the "missing middle." Hubs solve operational barriers — prior auth, copay, specialty pharmacy. But patients who have all the logistics handled still don't start therapy. The reason isn't access. It's fear, uncertainty, and the absence of someone who understands.

/ 5	**3.1**	Our program includes dedicated support for emotional and psychological barriers to treatment initiation — not just logistics.
/ 5	**3.2**	We can identify patients who are enrolled but hesitating (not filling, not scheduling, going silent) and intervene proactively.
/ 5	**3.3**	Patients have access to someone who has personally experienced the same treatment or condition — not just a nurse educator or call center agent.
/ 5	**3.4**	We have a documented process for addressing patient fears about side effects, lifestyle impact, and treatment commitment.
/ 5	**3.5**	We measure patient confidence and sentiment — not just enrollment numbers and call volumes.

SECTION 3 TOTAL / 25

4 Peer Connection & Mentorship

Can patients talk to someone who has been through what they're about to go through?

> 71% of patients who connect with a peer mentor say the experience was critical to their decision to initiate or continue treatment. Peer connection is the single highest-leverage intervention for converting enrolled-but-hesitant patients.

/ 5	**4.1**	Our program offers real-time, one-to-one connections between prospective patients and people who have completed the same treatment.
/ 5	**4.2**	Patients can access peer support when they need it (on-demand or same-day), not on a scheduled call weeks out.
/ 5	**4.3**	Mentors are matched based on condition, treatment, demographics, or experience — not randomly assigned.
/ 5	**4.4**	Our peer or mentor program is scalable across multiple brands, disease states, or therapy areas.
/ 5	**4.5**	We track outcomes from peer interactions: connection rate, impact on treatment decisions, progression to next step.

SECTION 4 TOTAL / 25

5 Adherence & Persistence Monitoring

Do you know what happens after the first fill?

> Average adherence rates for chronic therapies hover around 50%. Patients who disengage in the first 90 days are the hardest to recover.

/ 5	**5.1**	We track adherence and persistence beyond initial fill — at 30, 60, 90, and 180+ day intervals.
/ 5	**5.2**	We have automated triggers that identify patients at risk of discontinuation (missed refills, declining engagement).
/ 5	**5.3**	Patients receive ongoing support after treatment initiation — not just during enrollment and the first 30 days.
/ 5	**5.4**	Our adherence interventions include human connection (not just automated refill reminders or SMS nudges).
/ 5	**5.5**	We can quantify the impact of our support program on days on therapy and persistence rates.

SECTION 5 TOTAL / 25

6 Data, Analytics & ROI Measurement

Can you prove your program works — with numbers that matter?

> "Patient engagement" as a metric is dead. Leadership wants treatment starts, adherence rates, days on therapy, and revenue impact.

/ 5	**6.1**	We measure program impact using commercial outcomes (treatment starts, adherence, days on therapy) — not just engagement metrics.
/ 5	**6.2**	We can produce an ROI analysis that ties program investment to incremental revenue.
/ 5	**6.3**	We capture and analyze patient sentiment data — what patients say about their experience, concerns, and decision drivers.
/ 5	**6.4**	We have real-time dashboards or reporting (not quarterly reports).
/ 5	**6.5**	We use data from patient interactions to improve our program continuously.

SECTION 6 TOTAL / 25

7 Compliance & Scalability

Can you scale without scaling risk?

> Compliance isn't a feature — it's a prerequisite. The question is whether you can innovate within the guardrails.

/ 5	**7.1**	Our patient support platform is HIPAA-compliant with SOC 2 and/or ISO 27001 certification.
/ 5	**7.2**	All patient interactions are logged with audit trails accessible to compliance teams.
/ 5	**7.3**	Our program can be deployed across multiple brands or indications without rebuilding infrastructure.

/ 5 **7.4** We can launch a new support program for a new brand or indication in under 4 months.

/ 5 **7.5** Adverse event reporting and safety monitoring are built into patient interaction workflows.

SECTION 7 TOTAL / 25

Σ Your Total Score

SECTION	YOUR SCORE
1. Awareness & Discovery	_____ / 25
2. Enrollment & Onboarding	_____ / 25
3. Emotional Readiness & Psych Support	_____ / 25
4. Peer Connection & Mentorship	_____ / 25
5. Adherence & Persistence	_____ / 25
6. Data, Analytics & ROI	_____ / 25
7. Compliance & Scalability	_____ / 25
TOTAL	**_____ / 175**

140 – 175

Best-in-Class

Your program addresses the full patient journey. Focus on optimization: refining targeting, deepening analytics, and expanding across indications.

Your move: Benchmark against your own historical data. Where has improvement plateaued?

105 – 139

Strong Foundation, Visible Gaps

Solid operational infrastructure, but specific areas where patients slip through. Most programs in this range score low on Sections 3 and 4 — emotional readiness and peer connection. This is the "missing middle."

Your move: Your two lowest-scoring sections are your highest-ROI investment areas. A 10-point improvement there correlates with measurable gains in starts and adherence.

70 – 104

Functional but Leaking

Your program exists but is losing patients at multiple points. Measurement is likely focused on activity (calls, emails) rather than outcomes (starts, adherence, days on therapy).

Your move: Pick the two sections with the lowest scores and focus there. If Sections 3 and 4 are both below 15, that's your biggest single opportunity.

Below 70

Rebuilding Opportunity

Significant structural gaps — but also substantial opportunity. Even moderate improvements from this baseline produce outsized results.

Your move: Start with Sections 1 and 2 (Awareness and Enrollment). Then address Section 3 — the most under-invested area in the industry.

The Pattern We See in the Data

WHAT PATIENTS GET	WHAT PATIENTS NEED
Prior authorization support	Someone who's been through it
Copay assistance	Confidence the treatment is worth the fear
Nurse educator calls	A real conversation with a real patient
Automated refill reminders	Reassurance during the hardest moments

The industry spends billions on the left column. The right column is where decisions actually happen.

68% of connected patients take the next step in their treatment journey

71% report mentorship was critical to their treatment decision

73% mentor connection rate (vs. 10–20% for traditional support)

90% increase in confidence in treatment decisions

133.5 more days on therapy compared to unsupported patients

How to Use This With Your Team

1. **Score independently.** Have 3–5 stakeholders each complete the scorecard without comparing answers.
2. **Compare section scores.** Where do people agree? Where do they disagree? Disagreements reveal blind spots.
3. **Identify the 2 weakest sections.** These are your highest-ROI investment areas.
4. **Set a 90-day target.** Pick 3–5 items within your weakest sections and define what a 1-point improvement looks like.
5. **Re-score in 90 days.** Track progress. Use the scorecard as a recurring diagnostic.

Building the Business Case

THE PROBLEM

Patients enroll but don't start. Your program handles logistics well but doesn't address the psychological barriers — fear, doubt, lack of relatable support — that cause patients to stall.

THE COST

Every patient who enrolls but doesn't start represents lost therapy revenue, wasted acquisition spend, and reduced program ROI. For specialty therapies, a single patient's annual therapy value often exceeds $50,000.

THE SOLUTION

Peer-to-peer mentorship — connecting hesitant patients with trained mentors who've completed the same treatment. 68% more treatment starts. 29% better adherence. 133.5 more days on therapy.

If your program enrolls **1,000 patients per year** and **40% don't start therapy**, closing even half that gap with peer mentorship means **200 additional treatment starts**. At **$50,000 average annual therapy value**, that's **$10M in recovered revenue**.

See Where Peer Mentorship Fits in Your Program

Book a 30-minute strategy session. We'll walk through your scorecard results, show you where peer mentorship fits in your patient journey, and share benchmarks from similar programs.

68%	29%	<4mo
MORE TREATMENT STARTS	BETTER ADHERENCE	IMPLEMENTATION

Book a Strategy Session

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Data: PatientPartner platform studies (2023–2026), Phreesia/GoodRx PSP utilization research, IQVIA Four Forces (2026), Pharmaceutical Commerce.

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